

Clinical Handbook for Medical Students

Condition-Based Clinical Pathways and Management

Compiled from Clinical Guidelines and Condition Markdown Resources

This handbook is written in formal prose for teaching and bedside learning. Abbreviations are minimized and expanded in context to make the material easier to read and interpret during clinical training.

Condition 1: PV BLEEDING

Clinical presentation should be assessed systematically at first contact. Typical presenting complaints in the source material include Per Vaginal Bleeding. Important associated clinical features include with sign of shock(Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, blood pressure- (90mmHg) anuria, blood pressure - SBP <90mmHg; NO Local bleeding lesions; WITHOUT signs of shock(Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, blood pressure- (90mmHg) anuria, blood pressure - SBP <90mmHg & Hemodynamically stable. At this stage, the clinician should establish physiological stability and identify red-flag findings that require immediate escalation.

Diagnostic work-up should proceed in a staged manner so that management decisions are supported by objective findings. Recommended initial investigations include full haemogram + Pelvic ultrasound as you continue fluid resuscitation; full haemogram + OBS ultrasound as you continue fluid resuscitation; full haemogram + Pelvic ultrasound. Result patterns documented in the source include Normal full haemogram; Granulocytosis; Thrombocytopenia; Hb < 10 g/dl. When needed, additional tests may include Hormonal profile, urea, electrolytes, and creatinine,; Hormonal profile, urea, electrolytes, and creatinine, + liver function tests; Hormonal profile, urea, electrolytes, and creatinine, Coagulation profile + peripheral blood film. These findings are used to separate mild, moderate, and severe pathway branches and to guide urgency of intervention.

Differential diagnosis and pathway selection should be explicitly documented before treatment is finalized. Diagnostic formulations recorded in the source include Probable dx: SHOCK with Abnormal Uterine Bleeding ? Cause; Probable dx: SHOCK with Abnormal Uterine Bleeding ? Cause , ITP, VWD,; Probable dx: SHOCK with Abnormal Uterine Bleeding ? Cause, Anemia; Probable dx: Abortion with SHOCK; Probable dx: Abortion with SHOCK, DIC. Common management plans include Admit + Gynecological Review; Admit for Gynecological Review IF ACTIVELY BLEEDING If not bleeding see as outpatient & return for follow-up in GOPC. Where uncertainty remains, senior review and repeated clinical assessment are essential to avoid delayed escalation.

Therapeutic care should combine supportive measures with targeted treatment based on the selected pathway branch. Supportive interventions described in the source include See appendix (x) for hypovolemic shock resuscitation; Reassure patient, Start normal saline if actively bleeding as you consult. Treatment options include Tranexamic acid 1g intravenous tds, intravenous Ceftriaxone 50mg/kg, intravenous Metronidazole 7.5mg/kg; OUT PATIENT:Tranexamic acid 1g PO tds, outpatient Cefuroxime 50mg/kg,PO Metronidazole 7.5mg/kg, IN PATIENT Tranexamic acid 1g intravenous tds intravenous Ceftriaxone 50mg/kg, intravenous Metronidazole 7.5mg/kg. Example brand references provided in the source include Hemsamic 50mg inj, Rocephin 1gm,Flagyl infusion; Hemsamic 500mg caps, Axacef 500mg tab, Flagyl 400mg tablets. Final prescribing and dose decisions should always follow local formularies, patient-specific contraindications, and consultant supervision.

Condition 2: Fever and Elevated Body Temperature

Clinical presentation should be assessed systematically at first contact. Typical presenting complaints in the source material include Hotness of Body. Important associated clinical features include No other sign; Hallucinations/delusions/abnormal posturing; Convulsions with no other sign; Convulsions with no other sign in not a known convulsive. At this stage, the clinician should establish physiological stability and identify red-flag findings that require immediate escalation.

Diagnostic work-up should proceed in a staged manner so that management decisions are supported by objective findings. Recommended initial investigations include full haemogram + malaria AG; full haemogram + Malaria Ag+ urea, electrolytes, and creatinine + random blood glucose; full haemogram + Malaria Ag+ urea, electrolytes, and creatinine + random blood glucose (consider LP if no signs of raised ICP); full haemogram. Result patterns documented in the source include Normal full haemogram+ Malaria negative; Granulocytosis + malaria negative; lymphocytosis+ malaria negative; hb< 10g/dl + malaria negative. When needed, additional tests may include peripheral blood film+ Reticulocyte count+ HIV+group and crossmatch; liver function tests + peripheral blood film; liver function tests + peripheral blood film+ group and crossmatch+ reticulocyte count. These findings are used to separate mild, moderate, and severe pathway branches and to guide urgency of intervention.

Differential diagnosis and pathway selection should be explicitly documented before treatment is finalized. Diagnostic formulations recorded in the source include Acute febrile illness R/O viral, atypical infections; acute bacterial infection; acute viral infection; anaemia + acute febrile illness; Malaria. Common management plans include see as OUT PT ,return for follow-up 7 days; admit patient for further work up and possible transfusion; see as an outpatient and return for follow-up 3 days; admit patient for parenteral treatment. Where uncertainty remains, senior review and repeated clinical assessment are essential to avoid delayed escalation.

Therapeutic care should combine supportive measures with targeted treatment based on the selected pathway branch. Supportive interventions described in the source include Expose patient , tepid sponge; Expose patient , tepid sponge , secure an intravenous line neck support, reassure the relatives; Expose patient , tepid sponge , secure an intravenous line neck support, reassure the relatives start ringers lactate as you consult. Treatment options include no; amoxycillin+ clavulanate over 12years 375-750mg, below 12 years 25-50mg/kg/day, paracetamol 10mg/kg/dose; paracetamol 10mg/kg/dose, ceftriaxone 50mg/kg/dose, transfuse if symptomatic, ranferon; amoxycillin+ clavulanate over 12years 375-750mg, below 12 years 25-50mg/kg/day, paracetamol 10mg/kg/dose, artemether/lumefantrine per kg body weight. Example brand references provided in the source include fleming, paralief for children, panadol advance; Rocephin,paraconica 1g; fleming, paralief for children, panadol advance, ACTM 20/120. Final prescribing and dose decisions should always follow local formularies, patient-specific contraindications, and consultant supervision.

Condition 3: Elevated Blood Pressures in Pregnancy

Clinical presentation should be assessed systematically at first contact. Typical presenting complaints in the source material include ELEVATED blood pressure $\geq 140/90$ mmHg; blood pressure reading Syst:140-179/90- 109 OR isolated 180/110 regardless of S/S; blood pressure reading Syst:140-179 or 90- 109 mmHG. Important associated clinical features include WITH ANY OF:signs of target organ damage (Severe headache with blurring of vision, Epigastric pain, Oliguria , signs , liver tenderness; WITHOUT ANY OF:signs of target organ damage (Severe headache with blurring of vision, Epigastric pain, Oliguria , signs , liver tenderness. At this stage, the clinician should establish physiological stability and identify red-flag findings that require immediate escalation.

Diagnostic work-up should proceed in a staged manner so that management decisions are supported by objective findings. Recommended initial investigations include full haemogram + Urinalysis + urea, electrolytes, and creatinine + liver function tests + Coagulation profile; full haemogram + urea, electrolytes, and creatinine + liver function tests. Result patterns documented in the source include Normal full haemogram + Normal Transaminases + Normal urea, electrolytes, and creatinine + Normal coagulation profile + No Proteinuria; Hb <10 g/dl + Normal Transaminases + Normal urea, electrolytes, and creatinine + Normal coagulation profile + No Proteinuria; Thrombocytopenia < 100 + Normal Transaminases + Normal urea, electrolytes, and creatinine + Normal coagulation profile + No Proteinuria; Normal full haemogram + Elevated Transaminases + Normal urea, electrolytes, and creatinine + Normal coagulation profile + No Proteinuria. When needed, additional tests may include Obstetric ultrasound + Uric acid + peripheral blood film; Obstetric scan; Obstetric scan, peripheral blood film. These findings are used to separate mild, moderate, and severe pathway branches and to guide urgency of intervention.

Differential diagnosis and pathway selection should be explicitly documented before treatment is finalized. Diagnostic formulations recorded in the source include Severe Pre - Eclampsia; Gestational hypertension/Mild Pre Eclampsia; Gestational hypertension + Anaemia/ Mild Pre Eclampsia; Severe Pre eclampsia; Severe Pre eclampsia , Anaemia. Common management plans include Admit and call gynaecologist immediately. Where uncertainty remains, senior review and repeated clinical assessment are essential to avoid delayed escalation.

Therapeutic care should combine supportive measures with targeted treatment based on the selected pathway branch. Supportive interventions described in the source include reassure patient, place the patient in left lateral position, restrict fluids to 80mls per hr of ringers lactate avoid DNS or Dextrose.; reassure patient, place the patient in left lateral position.; reassure patient, place the patient in left lateral position,. Treatment options include if blood pressure > 160/110 mm/hg give intravenous hydralazine 5mg slowly over 10 minutes , Give loading 4 GM of 20% MGSO4 over 15 minutes then continue with 1 GM hourly for 24 hours as you monitor the respiratory rate, urine output and patellar reflexes. And continue for 24 hours after delivery whichever comes first.; Methyldopa 250 mg TID; Methyldopa 250mg TID; Methyldopa 250mgTID. Example brand references provided in the source include Apresolin 20mg inj, Nalepsin 4gm injection; Aldomet 250mg tablets; Aldomet 205mg tablets. Final prescribing and dose decisions should always follow local formularies, patient-specific contraindications, and consultant supervision.

Condition 4: HEAD INJURY

Clinical presentation should be assessed systematically at first contact. Typical presenting complaints in the source material include Trauma to the head. Important associated clinical features include WITH:-Convulsions, Confusion, Ottorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs , extremities of age < 5 years > 60 years; Regardless of:-Convulsions, Confusion, Ottorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs extremities of age < 5 years > 60 years; Regardless of: -Convulsions, Confusion, Ottorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs extremities of age < 5 years > 60 years; WITHOUT:-Convulsions, Confusion, Ottorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs , extremities of age < 5 years > 60 years. At this stage, the clinician should establish physiological stability and identify red-flag findings that require immediate escalation.

Diagnostic work-up should proceed in a staged manner so that management decisions are supported by objective findings. Recommended initial investigations include Order an urgent computed tomography

scan Head,; Skull X-Ray. When needed, additional tests may include urea, electrolytes, and creatinine, random blood glucose, LFTS full haemogram,Alcohol toxicity levels.; urea, electrolytes, and creatinine, random blood glucose, LFTS full haemogram, Alcohol toxicity levels. These findings are used to separate mild, moderate, and severe pathway branches and to guide urgency of intervention.

Differential diagnosis and pathway selection should be explicitly documented before treatment is finalized. Diagnostic formulations recorded in the source include Dx- Mild Head injury; Dx- Moderate Head injury; Dx- severe Head injury; Dx: Mild Head Injury; Dx: Moderate Head Injury. Common management plans include Admit and immediate surgical consult; Admit for Observation. Where uncertainty remains, senior review and repeated clinical assessment are essential to avoid delayed escalation.

Therapeutic care should combine supportive measures with targeted treatment based on the selected pathway branch. Supportive interventions described in the source include Assess Airway , Circulation, Breathing, Disability and Exposure(ACBDE) Arrest Bleeding(If Present) , if in shock see appendix (x). Treatment options include intravenous ceftriaxone 100mg/kg/dose , Paracetamol 10mg/kg/dose, Phenytoin 20mg/kg to run over 15 mins with ECG & blood pressure monitoring, TT 0.5mg IM SAT in > 5 years old. Example brand references provided in the source include Rocephin 2gm,Paraconica 1gm, Epanutin 250mg inj, Tetenus injection. Final prescribing and dose decisions should always follow local formularies, patient-specific contraindications, and consultant supervision.

Condition 5: ABDOMINAL INJURY

Clinical presentation should be assessed systematically at first contact. Typical presenting complaints in the source material include Abdominal Injury. Important associated clinical features include WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, blood pressure- (90mmHg) anuria, blood pressure - SBP <90mmHg; WITHOUT signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, blood pressure- (90mmHg) anuria, blood pressure - SBP <90mmHg. At this stage, the clinician should establish physiological stability and identify red-flag findings that require immediate escalation.

Diagnostic work-up should proceed in a staged manner so that management decisions are supported by objective findings. Recommended initial investigations include Focused Assessment Sonography in Trauma (FAST ultrasound) as you consult. When needed, additional tests may include full haemogram + urea, electrolytes, and creatinine + group and crossmatch + liver function tests; full haemogram + urea, electrolytes, and creatinine + liver function tests + group and crossmatch. These findings are used to separate mild, moderate, and severe pathway branches and to guide urgency of intervention.

Differential diagnosis and pathway selection should be explicitly documented before treatment is finalized. Diagnostic formulations recorded in the source include Blunt Abdominal injury with SHOCK; Penetrating Abdominal injury with SHOCK; Blunt Abdominal injury; Penetrating Abdominal injury. Common management plans include Admit and immediate surgical consult; Admit for observation. Where uncertainty remains, senior review and repeated clinical assessment are essential to avoid delayed escalation.

Therapeutic care should combine supportive measures with targeted treatment based on the selected pathway branch. Supportive interventions described in the source include See appendix (x) for

hypovolemic shock resuscitation; See appendix (x) for hypovolemic shock resuscitation, DO NOT REMOVE any impinged penetrating object, if no impinged object PACK WITH STERILE GAUZE; Reassure patient, maintenance IVF,. Treatment options include intravenous ceftriaxone 50mg/kg/dose, Iv Metronidazole 7.5mg/kg/dose, Paracetamol 10mg/kg/dose; intravenous ceftriaxone 50mg/kg/dose, Iv Metronidazole 7.5mg/kg/dose, Paracetamol 10mg/kg/dose , TT 0.5mg IM SAT in > 5 years old. Example brand references provided in the source include Rocephin 1gm injection, Flagyl infusion, paraconica 1gm; Rocephin 1gm injection, Flagyl infusion, paraconica 1gm tetanus injection. Final prescribing and dose decisions should always follow local formularies, patient-specific contraindications, and consultant supervision.

Condition 6: CHEST TRAUMA

Clinical presentation should be assessed systematically at first contact. Typical presenting complaints in the source material include Trauma to the Chest + Difficulty in breathing.. Important associated clinical features include WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, blood pressure- (90mmHg) anuria, blood pressure - SBP <90mmHg; WITHOUT signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, blood pressure- (90mmHg) anuria, blood pressure - SBP <90mmHg. At this stage, the clinician should establish physiological stability and identify red-flag findings that require immediate escalation.

Diagnostic work-up should proceed in a staged manner so that management decisions are supported by objective findings. Recommended initial investigations include chest x-ray. When needed, additional tests may include full haemogram, urea, electrolytes, and creatinine. Abdominal scan especially if # is in 11th and 12th ribs or with associated abdominal tenderness. These findings are used to separate mild, moderate, and severe pathway branches and to guide urgency of intervention.

Differential diagnosis and pathway selection should be explicitly documented before treatment is finalized. Diagnostic formulations recorded in the source include Dx- Blunt chest trauma in shock.; Dx- Blunt chest trauma; Dx- Blunt chest trauma with pneumothorax; Dx- Blunt chest trauma with hemothorax. Common management plans include Admit and consult immediately. Where uncertainty remains, senior review and repeated clinical assessment are essential to avoid delayed escalation.

Therapeutic care should combine supportive measures with targeted treatment based on the selected pathway branch. Supportive interventions described in the source include See appendix (x) for hypovolemic shock resuscitation,give oxygen, Do Assess and manage Airway, Circulation, Breathing, Disability. As you consult .IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK; Reassure patient, maintenance IVF,.IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK; See appendix (x) for hypovolemic shock resuscitation,give oxygen, Do Assess and manage Airway, Circulation, Breathing, Disability,insert a large bore cannula at 2nd intercostal space mid clavicular line,.IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK. Treatment options include intravenous ceftriaxone 50mg/kg/dose, Iv Metronidazole 7.5mg/kg/dose, Paracetamol 10mg/kg/dose. Example brand references provided in the source include Rocephin 1gm injection, Flagyl infusion, Paraconica 1gm injection. Final prescribing and dose decisions should always follow local formularies, patient-specific contraindications, and consultant supervision.

Condition 7: Epigastric PAIN

Clinical presentation should be assessed systematically at first contact. Typical presenting complaints in the source material include EPIGASTRIC PAIN. Important associated clinical features include bloody Vomitus WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, blood pressure- (90mmHg) anuria, blood pressure - SBP <90mmHg; bloody Vomitus WITHOUT signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, blood pressure- (90mmHg) anuria, blood pressure - SBP <90mmHg; Non Bloody VOMITUS, WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, blood pressure- (90mmHg) anuria, blood pressure - SBP <90mmHg; Non bloody Vomitus WITHOUT signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, blood pressure- (90mmHg) anuria, blood pressure - SBP <90mmHg. At this stage, the clinician should establish physiological stability and identify red-flag findings that require immediate escalation.

Diagnostic work-up should proceed in a staged manner so that management decisions are supported by objective findings. Recommended initial investigations include full haemogram + urea, electrolytes, and creatinine + liver function tests + H- Pylori Ag; full haemogram+ H- Pylori Ag. Result patterns documented in the source include Normal full haemogram + Normal Transaminases + Normal urea, electrolytes, and creatinine + H-Pylori -VE; Hb <10 g/dl + Normal Transaminases + Normal urea, electrolytes, and creatinine+ H-Pylori -VE; Thrombocytopenia < 100+ Normal Transaminases + Normal urea, electrolytes, and creatinine + H-Pylori -VE; Normal full haemogram + Elevated Transaminases + Normal urea, electrolytes, and creatinine + H-Pylori -VE. When needed, additional tests may include Endoscopy, group and crossmatch; Endoscopy ,coagulation profile, group and crossmatch; Endoscopy ,coagulation profile, Abdominal ultrasound, group and crossmatch. These findings are used to separate mild, moderate, and severe pathway branches and to guide urgency of intervention.

Differential diagnosis and pathway selection should be explicitly documented before treatment is finalized. Diagnostic formulations recorded in the source include probable dx- Upper GI bleeding. With shock; probable dx- Upper GI bleeding, Anaemia With shock; Probable dx- Upper GI bleeding with shock R/O a coagulopathy; probable dx- Liver failure, oesophageal varices.; probable dx- Liver failure, oesophageal varices. Coagulopathy.. Common management plans include Admit and urgent surgical consult; assess, see as an OPD and a return for follow-up of 7 days. Where uncertainty remains, senior review and repeated clinical assessment are essential to avoid delayed escalation.

Therapeutic care should combine supportive measures with targeted treatment based on the selected pathway branch. Supportive interventions described in the source include See appendix (x) for hypovolemic shock resuscitation; start intravenous normal saline as you consult; start on pain relieve. Treatment options include start on fluid resuscitation (see appendix x), iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT EXCEED 500MG > 12 years 500mg BID intravenous Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg TID intravenous Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ,Trenexamic 10mg/kg/dose/TID; Start on, iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT EXCEED 500MG > 12 years 500mg BID intravenous Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg TID intravenous Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ,Tranexamic acid 10mg/kg/dose TID; start on fluid resuscitation (see appendix x), iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT EXCEED 500MG > 12 years 500mg BID intravenous Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg TID intravenous Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ; > 12 years esomeprazole 20mg BID /day , paed 3.5kg : 2.5mg P.O /day , 3.5kg-7.5kg: 5mg PO /day, > 7.5 kg 10mg PO /day. Example brand references provided in the source include Klacid 500mg injection,Augmentin 1.2gm injection, Zantac 50mg inj, Hemsamic 500mg injection; Klacid 500mg injection,Augmentin 1.2gm injection, Zantac 50mg inj; Klacid 500mg injection,Augmentin

1.2gm injection, Zantac 50mg inj, hemsamic 500mg inj. Final prescribing and dose decisions should always follow local formularies, patient-specific contraindications, and consultant supervision.

Condition 8: WHEEZE

Clinical presentation should be assessed systematically at first contact. Typical presenting complaints in the source material include Wheezing with Difficulty in Breathing. Important associated clinical features include WITH ANY OF: While walking, Talks in Alertness, Can lie down, Sentences May be agitated, Increased Respiratory rate, no use of accessory muscles, wheeze often only end expiratory, Pulse Rate <100, Pulsus paradoxus, Absent/ <10 mm Hg; WITH ANY OF: While at rest (infant—softer, shorter cry, difficulty feeding), Commonly uses accessory muscles, wheeze Loud throughout exhalation, Pulse Rate 100-120 Age 2 -12Mo <160/min, 1-2 Years <120/min 2-8 years <110/min, Pulsus paradoxus May be present 10–25 mm Hg; WITH ANY OF: While at rest (infant—stops feeding), Sits upright, Words Usually agitated, wheeze loud throughout inhalation and exhalation, Pulse Rate >120, Pulsus paradoxus, Often present . 25 mm Hg (adult), 20–40 mm Hg (child); WITH: Drowsy / Confused, paradoxical thoracoabdominal movement, absence of wheeze, Bradycardia, absence of pulsus paradoxus. At this stage, the clinician should establish physiological stability and identify red-flag findings that require immediate escalation.

Diagnostic work-up should proceed in a staged manner so that management decisions are supported by objective findings. Recommended initial investigations include full haemogram; full haemogram + urea, electrolytes, and creatinine; full haemogram + urea, electrolytes, and creatinine + blood gas analyses. These findings are used to separate mild, moderate, and severe pathway branches and to guide urgency of intervention.

Differential diagnosis and pathway selection should be explicitly documented before treatment is finalized. Diagnostic formulations recorded in the source include MILD ASTHMATIC ATTACK; MODERATE ASTHMATIC ATTACK; SEVERE ASTHMATIC ATTACK; ACTUAL/ IMPENDING RESPIRATORY FAILURE. Common management plans include TREAT AS outpatient & return for follow-up 24HRS.; ADMIT & CALL PHYSICIAN IMMEDIATELY; ADMIT IN CRITICAL CARE & CALL PHYSICIAN IMMEDIATELY. Where uncertainty remains, senior review and repeated clinical assessment are essential to avoid delayed escalation.

Therapeutic care should combine supportive measures with targeted treatment based on the selected pathway branch. Supportive interventions described in the source include Reassure patient/ guardian, administer oxygen to maintain SPO2 > 90%, Give health education See appendix (XII); INTUBATE PATIENT Reassure patient/ guardian, administer oxygen to maintain SPO2 > 90%, Give health education See appendix (XII). Treatment options include Nebulize with Salbutamol 1Mo - 11 yrs 2.5mg, > 11 years 5mg can be repeated up to 3 doses in the 1st hour + Prednisolone 1mg/kg daily for 1/52; Nebulize with Salbutamol 1Mo - 11 yrs 2.5mg, > 11 years 5mg can be repeated up to 3 doses in the 1st hour + intravenous Ipratropium 200mg + intravenous Hydrocortisone 50mg(1-5 yrs), 100mg (6-12 yrs),. Example brand references provided in the source include Ventolin nebulizing solution, prednisolone 5mg tablets; Ventolin nebulizing solution, Atrovent 250mg nebules, Hydrocortison 100mg inj. Final prescribing and dose decisions should always follow local formularies, patient-specific contraindications, and consultant supervision.

Condition 9: BURNS

Clinical presentation should be assessed systematically at first contact. Typical presenting complaints in the source material include PATIENT WITH BURNS. Important associated clinical features include Partial thickness burns greater than 10% total body surface area (total body surface area).

2. Burns that involve the face, hands, feet, genitalia, perineum, or major joints. 3. Third degree burns in any age group. 4. Electrical burns, including lightning injury. 5. Chemical burns. 6. Inhalation injury. 7. Burn injury in patients with preexisting medical disorders that could complicate management, prolong recovery, or affect mortality. 8. Any patient with burns and concomitant trauma (such as fractures) in which the burn injury poses the greatest risk of morbidity or mortality. . 9. Extremes of age <5 & >60 10. Burn injury in patients who will require special social, emotional, or rehabilitative intervention 12. Circumfrential burns. At this stage, the clinician should establish physiological stability and identify red-flag findings that require immediate escalation.

Diagnostic work-up should proceed in a staged manner so that management decisions are supported by objective findings. These findings are used to separate mild, moderate, and severe pathway branches and to guide urgency of intervention.

Differential diagnosis and pathway selection should be explicitly documented before treatment is finalized. Diagnostic formulations recorded in the source include MILD BURN INJURY; MODERATE BURN INJURY; SEVERE BURN INJURY. Common management plans include Treat as outpatient & return for follow-up 3 days; Admit for Surgical review. Where uncertainty remains, senior review and repeated clinical assessment are essential to avoid delayed escalation.

Therapeutic care should combine supportive measures with targeted treatment based on the selected pathway branch. Supportive interventions described in the source include IN SEVERE BURNS DO ACBE remove all burned clothing, immerse the site in cold water for 30 minutes,douse with cool water, apply clean wraps on burned area,administer tetanus prophylaxis, gently cleanse the burn with 0.25% chlorhexidine solution, DO NOT use alcohol-based solutions,Gentle scrubbing will remove the loose necrotic tissue. Apply a thin layer of antibiotic cream (silver sulfadiazine). Dress the burn with petroleum gauze and dry gauze; IN SEVERE BURNS DO ACBE remove all burned clothing, immerse the site in cold water for 30 minutes,douse with cool water, apply clean wraps on burned area,administer tetanus prophylaxis, gently cleanse the burn with 0.25% chlorhexidine solution, DO NOT use alcohol-based solutions,Gentle scrubbing will remove the loose necrotic tissue. Apply a thin layer of antibiotic cream (silver sulfadiazine). Dress the burn with petroleum gauze and dry gauze SEE APPENDIX 12 FOR FLUID MANEGEMENT IN BURNS. Treatment options include Fluloxacillin 50mg/kg/dose + Paracetamol 10mg/kg/dose + silver sulphur diazine cream; intravenous Fluloxacillin 50mg/kg/dose + intravenous Metronidazole 7.5mg/kg + Pethidine 50mg IM + Tetracycline eye ointment in facial burns, Pethidine 50mg IM,Omeprazole 4mg/kg/dose,silver sulphur diazine cream; intravenous Fluloxacillin 50mg/kg/dose + intravenous Metronidazole 7.5mg/kg + Pethidine 50mg IM + Tetracycline eye ointment in facial burns, Pethidine 50mg IM,Omeprazole 4mg/kg/dose, silver sulphur diazine cream. Example brand references provided in the source include Adult Floxapen 500mg caps, Panadol advance, silverex 500gm, 25gm, Paed floxapen 125mg syrup, panadol baby and infant, silverex 500gm, 25gm; Floxapen 500mg inj, Flagyl infusion, pethidine 50mg, tetracycline eye ointment,losec 40mg injection,silverex cream 500gm. Final prescribing and dose decisions should always follow local formularies, patient-specific contraindications, and consultant supervision.

Condition 10: HYPONATREMIA

Clinical presentation should be assessed systematically at first contact. At this stage, the clinician should establish physiological stability and identify red-flag findings that require immediate escalation.

Diagnostic work-up should proceed in a staged manner so that management decisions are supported by objective findings. These findings are used to separate mild, moderate, and severe pathway branches

and to guide urgency of intervention.

Differential diagnosis and pathway selection should be explicitly documented before treatment is finalized. Where uncertainty remains, senior review and repeated clinical assessment are essential to avoid delayed escalation.

Therapeutic care should combine supportive measures with targeted treatment based on the selected pathway branch. Treatment options include (Serum Na < 130 mmol/L); signs and symptoms; nausea, malaise, lethargy, decreased level of consciousness, headache, if severe (seizures and coma), overt neurologic symptoms (<115 mEq/L); CLASSIFICATION. Final prescribing and dose decisions should always follow local formularies, patient-specific contraindications, and consultant supervision.

End of Handbook